MC-2202 Scan QR code to check report authenticity

Passport No : **LABORATORY TEST REPORT**

Patient Information Sample Information Client/Location Information Name : **Lyubochka Svetka** **Lab Id** Client Name : Sterling Accuris Buddy : **02232160XXXX**

Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected at : non SAWPL Collected on : 20-Feb-2023 08:53 Location : Approved on : 20-Feb-2023 11:09 Status : Final

Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : EDTA Blood Process At : Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Complete Blood Count

Test Result Unit Biological Ref. Interval Hemoglobin Colorimetric 14.5 g/dL 13.0 - 16.5

RBC Count Electrical impedance 4.79 million/cmm 4.5 - 5.5

Hematocrit Calculated 43.3 % 40 - 49

MCV Derived 90.3 fL 83 - 101

MCH Calculated 30.2 pg 27.1 - 32.5

MCHC Calculated 33.4 g/dL 32.5 - 36.7

RDW CV Calculated 13.60 % 11.6 - 14

Total WBC and Differential Count

WBC Count SF Cube cell analysis H **10570 /cmm** 4000 - 10000

Differential Count Absolute Count

Neutrophils Microscopic 73 % 40 - 80 **Lymphocytes** Microscopic **19 %** 20 - 40 **Eosinophils** Microscopic 02 % 1 - 6 **Monocytes** Microscopic 06 % 2 - 10 **Basophils** Microscopic 00 % 0 - 2 **7716 /cmm** 2000 - 6700 2008 /cmm 1100 - 3300 211 /cmm 00 - 400 634 /cmm 200 - 700 0 /cmm 0 - 100

Platelet Count Electrical impedance 150000 /cmm 150000 - 410000 **MPV** Calculated H **14.00 fL** 7.5 - 10.3

Peripheral Smear Examination

RBC Morphology Normochromic Normocytic

WBC Morphology WBCs Series Shows Normal Morphology

Platelets Morphology Platelets are adequate with normal morphology.

Parasites Malarial parasite is not detected.

Erythrocyte Sedimentation Rate

ESR Capillary photometry 7 mm/1hr 0 - 14

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Patient Information **Sample Information Client/Location Information**

Name : **Lyubochka Svetka** Client Name : Sterling Accuris Buddy **Lab Id : 02232160XXXX**

Registration on : 20-Feb-2023 09:10 :

Location non SAWPL

Sex/Age **Male / 41 Y** 01-Feb-1982 Collected at : Approved on : 20-Feb-2023 13:33 Status : Final

:

Ref. Id : : 20-Feb-2023 08:53 Printed On : 28-Feb-2023 10:26 Collected on

Sample Type : EDTA Blood,

Ref. By : : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Serum Process At

Blood Group

Test Result	Unit	Biological Ref. Interval
ABO Type "A"		
Rh (D) Type Positive		



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Patient Information Sample Information Client/Location Information Name **: Lyubochka Svetka** **Lab Id** Client Name

: Sterling Accuris Buddy **: 02232160XXXX**

Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Location : Approved on : 20-Feb-2023 11:29 Status : Final

Ref. Id **:** Printed On : 28-Feb-2023 10:26 Sample Type : Serum Process At

Ref. By **:** : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Lipid Profile

Test Result Unit Biological Ref. Interval Cholesterol mg/dL Desirable : <200

Cholesterol oxidase – Peroxidase method 189.0

Borderline High : 200-239 High : >240

Triglyceride H **mg/dL Normal : <150**

Ezymatic (Lipase/GK/GPO/POD) **168.0** **Very High : >500**

Borderline : 150-199 High : 200-499

High: >60.0 PTA/MgCl2 60.0 HDL Cholesterol mg/dL Low: <40.0 Direct measured **100.39**

Direct LDL H **mg/dL Optimal: <100 Near to above Optimal:**

100–129
Borderline High: 130-159
High: 160–189
Very High: =190

VLDL mg/dL 15 - 35 Calculated 33.60

CHOL/HDL Ratio Up to 5.0 Calculated 3.1

LDL/HDL Ratio Up to 3.5 Calculated 1.7

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Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Location : Approved on : 20-Feb-2023 11:45 Status : Final

Ref. Id **:** Printed On : 28-Feb-2023 10:26 Sample Type : Fluoride plasma Process At **:**

Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test Result Unit Biological Ref. Interval Fasting Blood Sugar H **mg/dL 74 - 106** GOD-POD **141.0**

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: Sterling Accuris Buddy **: 02232160XXXX**

Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Location : Approved on : 20-Feb-2023 11:33 Status : Final

Ref. Id **:** Printed On : 28-Feb-2023 10:26 Sample Type : EDTA Blood Process At

Ref. By **:** : 1. NRL SAWPL Gujarat Ahmedabad Paldi

HbA1c (Glycosylated Hemoglobin)

Test Result	Unit	Biological Ref. Interval	
HbA1c H **% For Screening: Diabetes: >6.5%**			
			Pre-Diabetes: 5.7% - 6.4%
			Non-Diabetes: < 5.7%
			For Diabetic Patient:
			Poor Control : > 7.0 %
			Good Control : 6.0-7.0 %
High Performance Liquid Chromatography **7.10**			
Mean Blood Glucose	mg/dL		
Calculated 157.07			

Explanation:-

- Total haemoglobin A1 c is continuously synthesized in the red blood cell throught its 120 days life span. The concentration of HBA1c in the cell reflects the average blood glucose concentration it encounters.
- The level of HBA1c increases proportionately in patients with uncontrolled diabetes. It reflects the average blood glucose concentration over an extended time period and remains unaffected by short-term fluctuations in blood glucose levels.
- The measurement of HbA1c can serve as a convenient test for evaluating the adequacy of diabetic control and in preventing various diabetic complications. Because the average half life of a red blood cell is sixty days, HbA1c has been accepted as a measurement which reflects the mean daily blood glucose concentration, better than fasting blood glucose determination, and the degree of carbohydrate imbalance over the preceding two months.
- It may also provide a better index of control of the diabetic patient without resorting to glucose loading procedures.

HbA1c assay Interferences:
Erroneous values might be obtained from samples with abnormally elevated quantities of other Haemoglobins as a result of either their simultaneous elution with HbA1c (HbF) or differences in their glycation from that of HbA (HbS).

Reference: ADA Guideline 2023

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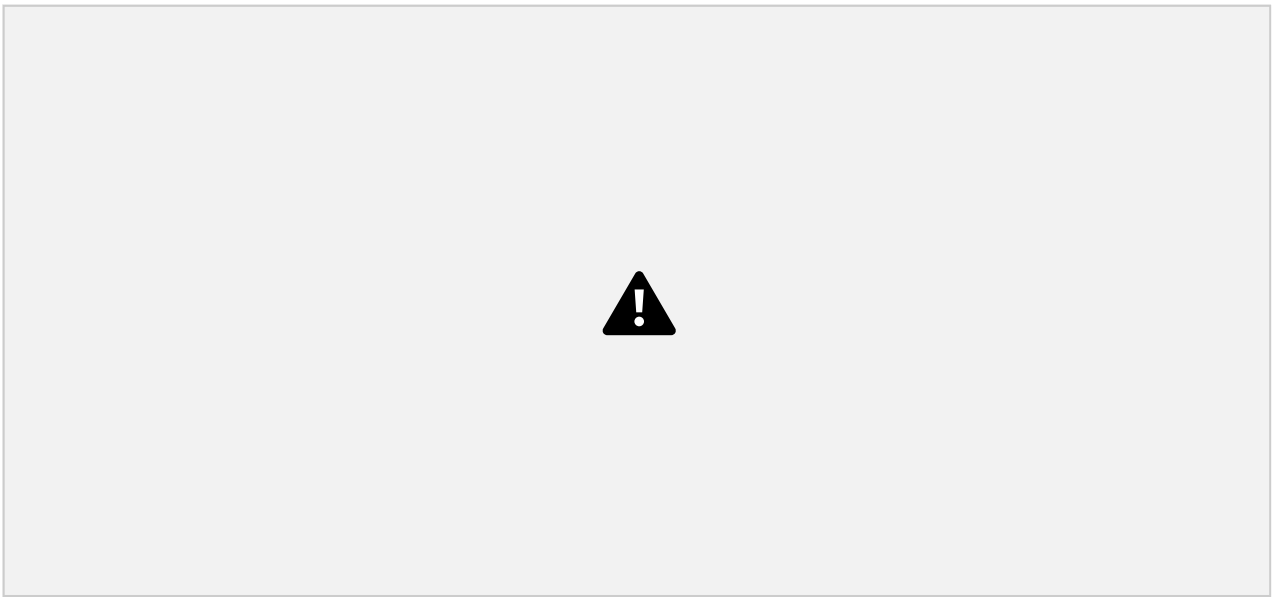
Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected Location : at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Approved on : 20-Feb-2023 11:38 Status : Final

Ref. Id **:** Printed On : 28-Feb-2023 10:26 Sample Type : Serum Process At : Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Thyroid Function Test

Test Result Unit Biological Ref. Interval T3 - Triiodothyronine ng/mL 0.58 - 1.59 Chemiluminescence 1.01

T4 - Thyroxine 7.84 mg/mL 4.87 - 11.72 TSH - Thyroid Stimulating Hormone microIU/mL 0.35 - 4.94 Chemiluminescence 0.8199



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: Sterling Accuris Buddy : **02232160XXXX**

Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected Location : at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Approved on : 20-Feb-2023 12:40 Status : Final

Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : Urine Process At

Ref. By : : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test Result Unit Biological Ref. Interval Immunoturbidimetric 10.50

Microalbumin (per urine volume) mg/L < 16.7

In random urine specimens, normal urinary albumin excretion is below 17 mg/g creatinine for males and below 25 mg/g creatinine for females.(3) Microalbuminuria is defined as an albumin:creatinine ratio of 17 to 299 for males and 25 to 299 for females.
A ratio of albumin:creatinine of 300 or higher is indicative of overt proteinuria.
Due to biologic variability, positive results should be confirmed by a second, first-morning random or 24-hour timed urine specimen. If there is discrepancy, a third specimen is recommended. When 2 out of 3 results are in the microalbuminuria range, this is evidence for incipient nephropathy and warrants increased efforts at glucose control, blood pressure control, and institution of therapy with an angiotensin-converting-enzyme (ACE) inhibitor (if the patient can tolerate it).

Reference :

1. Bennett PH, Haffner S, Kasiske BL, et al: Screening and management of microalbuminuria in patients with diabetes mellitus: recommendations to the Scientific Advisory Board of the National Kidney Foundation from an ad hoc committee of the Council on Diabetes Mellitus of the National Kidney Foundation. Am J Kidney Dis 1995;25:107-112
2. Krolewski AS, Laffel LM, Krolewski M, et al: Glycosylated hemoglobin and the risk of microalbuminuria in patients with insulin-dependent diabetes mellitus. N Engl J Med 1995;332:1251-1255
3. Zelmanovitz T, Gross JL, Oliveira JR, et al: The receiver operating characteristics curve in the evaluation of a random urine specimen as a screening test for diabetic nephropathy. Diabetes Care 1997;20:516-519

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Protein

Test Result Unit Biological Ref. Interval Total Protein g/dL 6.3 - 8.2 Copper tartrate to colour complex 7.00

Albumin g/dL 3.5 - 5.0 Bromocresol Green Method 4.20

Globulin g/dL 2.3 - 3.5 Calculated 2.80

A/G Ratio 1.3 - 1.7 Calculated 1.50

Bilirubin

Azobilirubin chromophores 0.70

Total Bilirubin mg/dL 0.2 - 1.3 Conjugated Bilirubin mg/dL 0.0 - 0.3 Cationic Mordant Binding 0.30

Cationic Mordant Binding 0.20

Unconjugated Bilirubin mg/dL 0.0 - 1.1 Calculated 0.20

Delta Bilirubin mg/dL 0.0 - 0.2

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Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : Serum Process At

Ref. By : : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Iron Studies

Test Result Unit Biological Ref. Interval Pyridyl azo Dye 103.00

Iron micro g/dL 49 - 181 Total Iron Binding Capacity (TIBC) 352.00 261 - 462 Transferrin

Saturation % Children : >16 Adult : 20 - 50 Calculated 29.26

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Patient Information **Sample Information** **Client/Location Information** Name : **Lyubochka Svetka** **Lab Id** Client Name

: Sterling Accuris Buddy : **02232160XXXX**

Immunoassay

Test Result Unit Biological Ref. Interval Homocysteine, Serum H **micromol/L 6.0 - 14.8**

Chemiluminescence **23.86**

Summary and Uses:

- Total Hcy is a thiol-containing amino acid, produced by the intracellular demethylation of methionine to cysteine.
- Elevated levels of t Hcy may be used to exclude or confirm deficiencies of vitamin B12 or folate. • It is recommended to test in patients using medications that interfere with folate status (methotrexate, antiepileptics), vegetarians without B12 supplementations, unexplained anemia, peripheral neuropathy or myleopathy, recurrent spontaneous abortions or infertility.
- Testing also recommended for patients 40 years of age with coronary artery disease to exclude homocystinuria.
- Elevations in tHcy levels have also been used as an independent risk factor of coronary or cerebral vascular disease. Treatment of moderate hyperhomocystinemia with folic acid supplementation for primary and secondary cardiovascular protection has met with inconsistent results and at present cannot be routinely recommended.

Limitations:

- The plasma must be seprated immediately on collection to avoid continuous synthesis of Hcy by red cells.
- Samples must be immediately stored on ice and serum centrifuged immediately before a complete clot is formed.
- Certain drugs, such as anticonvulsants, methotrexate, or nitrous oxide, may interfere with the assay.
- Cigarette smoking and coffee consumption increase tHcy levels.
- Intraindividual variability is approximately 8%; it can be as much as 25% in patients with hyperhomocystinemia.
- Generally, a single measurement of tHcy is considered adequate.

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Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : Serum Process At :

Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Biochemistry

Test Result Unit Biological Ref. Interval Creatinine, Serum mg/dL 0.66 - 1.25 Creatinine Amidohydrolase 0.83

Urea L **mg/dL 19.3 - 43.0** Urease, Colorimetric **18.0**

Blood Urea Nitrogen L **mg/dL 9.0 - 20.0** Calculated **8.41**

Uric Acid mg/dL 3.5 - 8.5 Uricase 4.90

Arsenazo III 9.10

Calcium mg/dL 8.4 - 10.2 **SGPT** U/L 0 - 50

UV with P5P, IFCC 48.0

SGOT U/L 17 - 59 UV with P5P 27.0

Electrolytes

Sodium (Na+) mmol/L 136 - 145 Direct- ISE 143.00

Direct- ISE 4.90

Potassium (K+) mmol/L 3.5 - 5.1 **Chloride (Cl-)** mmol/L 98 - 107 Direct- ISE 105.0

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Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected Location : at : non SAWPL Collected on : 20-Feb-2023 08:53 Approved on : 20-Feb-2023 12:33 Status : Final

Immunoassay

Test Result Unit Biological Ref. Interval 25(OH) Vitamin D ng/mL Deficiency : <10

CLIA **8.98** **Toxicity : >100**

Insufficiency : 10 - 30 Sufficiency : 30 - 100

Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol(vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.

Interpretation:

Increased In

- Vitamin D intoxication
- Excessive exposure to sunlight

Decreased In

- Malabsorption
- Steatorrhea
- Dietary osteomalacia, anticonvulsant osteomalacia
- Biliary and portal cirrhosis
- Thyrotoxicosis
- Pancreatic insufficiency
- Celiac disease
- Rickets
- Alzheimer disease

Limitations:

More recently, it has become clear that receptors for vitamin D are present in a wide variety of cells and that this hormone has biologic effects extending beyond the control of mineral metabolism. Vitamin D deficiency is not clear. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20–30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL. A recent study states that increasing mean baseline levels from 29 to 38 ng/mL was associated with a 50% lower risk for colon cancer and levels of 52 ng/mL with a 50% reduction in the incidence of breast cancer. It is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.

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Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected Location : at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Approved on : 20-Feb-2023 12:04 Status : Final

Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : Serum Process At
Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test Result Unit Biological Ref. Interval Vitamin B12 L **pg/mL 187 - 833** CLIA **< 148**

Vitamin B12 is essential in DNA synthesis, hematopoiesis, and CNS integrity.

Interpretation:

- **Increased In :** Chronic granulocytic leukemia , COPD and Chronic renal failure , Leukocytosis , Liver cell damage (hepatitis, cirrhosis) , Obesity and Severe CHF , Polycythemia vera , Protein malnutrition.
- **Decreased In :** Abnormalities of cobalamin transport or metabolism , Bacterial overgrowth , Crohn disease , Dietary deficiency (e.g. in vegetarians) , Diphyllobothrium (fish tapeworm) infestation , Gastric or small intestine surgery , Hypochlorhydria , Inflammatory bowel diseas , Intestinal malabsorption and Intrinsic factor deficiency

Limitations:

- Drugs such as chloral hydrate increase vitamin B12 levels. On the other hand , alcohol, aminosalicylic acid, anticonvulsants, ascorbic acid, cholestyramine, cimetidine, colchicines, metformin, neomycin, oral contraceptives, ranitidine, and triamterene decrease vitamin B12 levels. • The evaluation of macrocytic anemia requires measurements of both vitamin B12 and folate levels; ideally they should be measured simultaneously.
- Specimen collection soon after blood transfusion can falsely increase vitamin B12 levels.
- Patients taking vitamin B12 supplementation may have misleading results.
- A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

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Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Location : Approved on : 20-Feb-2023 11:38 Status : Final

Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : Serum Process At

Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Immunoassay

Test Result Unit Biological Ref. Interval PSA-Prostate Specific Antigen, Total 0.573 ng/mL 0 - 4

PSA is a glycoprotein that is expresses by both normal and neoplastic prostate tissue and is prostate tissue specific and not prostate cancer specific. PSA is constantly expressed in nearly all prastate cancers, although its level of expression on a percell basis is lower than in normal prostate epithelium. The absolute value of serum PSA is useful for determining the extent of prostate cancer and assessing the response to prostate cancer treatment; its use as a screening method to detect prostate cancer is also common.

Interpretation

Increased in

- Prostate disease (Cancer, Prostatitis, Benign prostatic hyperplasia, Acute urinary retention)
- Manipulations (Cystoscopy, Needle biopsy, Radiation therapy, Indwelling catheter, Prostatic massage)
- Transurethral resection
- Prostatic ischemia

Decreased in

- Castration
- Prostatectomy
- Radiation therapy
- Ejaculation withi 24 - 48 hours
- 5-alpha-reductase inhibitor reduces PSA by 50% after 6 months in men without cancer

Limitations

- PSA has been recommended by the American Cancer Society for use in conjunction with a DRE for early detection of prostate cancer starting at the age of 50 years for men with at least 10 year life expectancy
- PSA levels that are measured repeatedly over time may vary because of biologic variability where the true PSA level in a given man is different on different measurements.
- A change in PSA of >30% in man with a PSA initially below 2.0 ng/mL was likely to indicate a true change beyond normal random variation.

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Patient Information

Name **: Lyubochka Svetka**

Sex/Age **Male / 41 Y**

Lab Id Client Name **:** Sterling Accuris Buddy **: 02232160XXXX**

01-Feb-1982 : 09:10 Collected at **:** non SAWPL

Registration on : 20-Feb-2023 Location :

Approved on : 20-Feb-2023 12:06 Status : Final

Collected on **:** 20-Feb-2023 08:53

Ref. Id **:** Printed On : 28-Feb-2023 10:26 Sample Type : Serum

Ref. By **:** Process At : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Sample Information Client/Location Information

Immunoassay

Test Result Unit Biological Ref. Interval IgE H **IU/mL 0 - 87**

CLIA **492.30**

IgE mediates allergic and hypersensitivity reactions. There is a significant overlap in total IgE between allergic and nonallergic individuals.

Interpretation:

- Increased In
  - Atopic diseases
    - Exogenous asthama in approximately 60% of patients
    - Hay fever in approximately 30% of patients and Atopic eczema
  - Influenced by type of allergen, duration of stimulation. Presence of symptoms, and hyposensitization treatment
  - Parasitic diseases (e.g. ascariasis, visceral larva migrans, hookworm disease, schistosomiasis, Echinococcus infestation)
  - Monoclonal IgE myeloma

- Decreased In

- Hereditary deficiencies
- Acquired immunodeficiency
- Ataxia-telangiectasis
- Non-IgE myeloma

Limitations:

- A normal level of IgE in serum does not eliminate the possibility of allergic disease.
- Serum total IgE levels for the majority of individuals with IgE-mediated disease can be expected to be elevated compared to the reference range for healthy adults. However, not all allergic patients exhibit elevated serum total IgE levels.
- Since not all atopic reactions are IgE-mediated, a total IgE result in the reference range should always be interpreted in light of other clinical observations.
- Heterophilic antibodies in human serum can react with the immunoglobulins included in the assay components causing interference with in vitro immunoassays.

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Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : Serum Process At :

Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Test Result	Unit	Biological Ref. Interval	
Reactive : >1.0			Chemiluminescence
0.070			

HIV I & II Ab/Ag with P24 Ag S/Co Non Reactive : <1.0

Interpretation Non Reactive

Chemiluminescence

Reactive: >1.0 0.290 HBsAg S/Co Non Reactive : <1.0

Interpretation Non Reactive

Additional Information:

1. A NON REACTIVE result implies that no Anti HIV-1 or HIV -2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the "WINDOW PHASE" (before the development of detectable levels of antibodies).
2. A PROVISIONALITY REACTIVE / BORDERLINE REACTIVE result suggests possibility of HIV-1 or/and HIV-2 infection. However these results must be verified by confirmatory WESTERN BLOT / HIV PCR method before declaring the patient positive for HIV-1 or HIV-2 infection. 3. Very high levels of IgM Antibodies or Anti-HLA ABC and DR Antibodies can give false positive reaction.

**Pre & Post test counselling for HIV testing is responsibility of reffering Physician.

Dr. Siddharth Thummar **Dr. Sanjeev Shah** **Dr.Yash Shah** M.D. Pathology MD Path MD Path

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Ref. Id : Printed On : 28-Feb-2023 10:26 Sample Type : EDTA Blood Process At :

Ref. By : 1. NRL SAWPL Gujarat Ahmedabad Paldi

HB Electrophoresis By HPLC

Instrument Name: BIORAD VARIANT - II Haemoglobin Testing System

Test Result Unit Biological Ref. Interval Hb A L **84.4** % 96.8 - 97.8 Hb A2 2.8 % 2.2 - 3.2

P2 Peak 5.5 %

P3 Peak 5.2 %

Foetal Hb 0.3 % 0.0 - 1.0

Interpretation **Negative for typical beta thalassemia trait.**

Interpretation:

- All results have to be correlated with age and history of blood transfusion if there is history of blood transfusion in last 3 months, repeat testing after 3 month from last date of transfusion is recommended.
- In case of haemoglobinopathy, parents or family studies and councelling is advised.
- This test detects beta thalassaemia and haemoglobinopathies, DNA analysis is recommended to rule out alpha thalassaemia and silent carriers.
- Linearity range of HbF is 1-40%, However, values in excess of the reportable range have been provided for ease of interpretation. • Mild to moderate increase in fetal haemoglobiin can be seen in some acquired condition like pregnancy, megaloblastic anaemia, Throtoxicosis, Hypoxia, Chronic kidney disease, Recovering marrow, MDS, Aplastic anaemia, PNH, Medications (Hydrocyurea, Erythropoietin) ect. • P3 window-Above 10% is often indicative of either denatured froms of hemoglobins or may suggest a possibility of abnormal haemoglobin variant. Hence, repeat analysis with fresh sample or DNA studies is advised.
- P2 Window-Above 10% is indicative of either glycated haemoglonin requring correlation with diabetic staus or may suggest a possibility of abnormal haemoglobin variant further DNA studies for confirmation.

Dr. Hardik Modi **Dr. Sanjeev Shah** **Dr.Yash Shah** Hematopathologist (G-18097) MD Path MD Path

This is an Electronically Authenticated Report. # Referred Test

Bio-Rad CDM System PATIENT REPORT VII Inst. #1. SN-13939 V2_BThal

Patient Data **Analysis Data**

Sample ID: **022321600126** Analysis Performed: 02/20/2023 13:56:59 Patient ID: Injection Number: 2575
Name: Run Number: 95
Physician: Rack ID: 0001
Sex: Tube Number: 2
DOB: Report Generated: 02/20/2023 14:04:40 Comments: Operator ID:

Calibrated Retention Peak
Peak Name Area % Area % Time (min) Area
Unknown --- 0.1 1.00 2400

F	0.3	---	1.08	8321
Unknown	---	1.6	1.18	44109
P2	---	5.5	1.30	148807

P3	---	5.2	1.72	139183
Ao	---	84.4	2.34	2277592
A2	2.8	---	3.64	76999

Total Area: 2,697,411

F Concentration = 0.3 %
A2 Concentration = 2.8 %

Analysis comments:

45.0

37.5

30.0

%
22.5 15.0

1.3^{0}
1.1^{8}
1.7^{2}
3.6^{4}
7.5
1.0^{8}
1.0^{0}
–
–
2.3^{4}
A2$^{–}$
0.0
–
F$^{–}$
–
–

0 1 2 3 4 5 6 **Time (min.)**

MC-2202 Scan QR code to check report authenticity

Passport No : **LABORATORY TEST REPORT**

Patient Information Sample Information Client/Location Information Name **: Lyubochka Svetka** **Lab Id** Client Name

: Sterling Accuris Buddy **: 02232160XXXX**

Sex/Age **Male / 41 Y** 01-Feb-1982 : Registration on : 20-Feb-2023 09:10 Collected Location : at **:** non SAWPL Collected on **:** 20-Feb-2023 08:53 Approved on : 20-Feb-2023 11:12 Status : Final

Ref. Id **:** Printed On : 28-Feb-2023 10:26 Sample Type : Urine Process At

Ref. By **:** : 1. NRL SAWPL Gujarat Ahmedabad Paldi

Test	Result	Unit	Biological Ref. Interval
Physical & Chemical (Dip strip) examination			
Colour	Pale Yellow		Pale Yellow
Clearity	Clear		Clear
pH	6.0	Double indicator	4.6 - 8.0
Specific Gravity	1.030	Polyelectrolyte based reaction	1.005 - 1.030
Urine Glucose	**Present (+)**	GOD-POD	**Absent**
Protein error of indicators	Absent		
Urine Protein	Absent	Diazo reaction	Absent
Bilirubin	Absent	Modified Ehrlich reaction	Absent
Urobilinogen	Absent	Nitroprusside	Absent
Urine Ketone	Absent	Nitrite reaction	Absent
Nitrite	Absent		
Microscopic Examination			
Pus Cells	1-2		Absent
Red Cells	Nil	/hpf	0 - 2
Epithelial Cells	1-2	/hpf	.
Casts	Absent	/hpf	Absent
Crystals	Absent	/hpf	Absent
Amorphous Material	Absent		

---------- End Of Report ----------

DR.TEJASWINI DHOTE **Dr. Sanjeev Shah** **Dr.Yash Shah** M.D. Pathology MD Path MD Path

This is an Electronically Authenticated Report. #

Referred Test